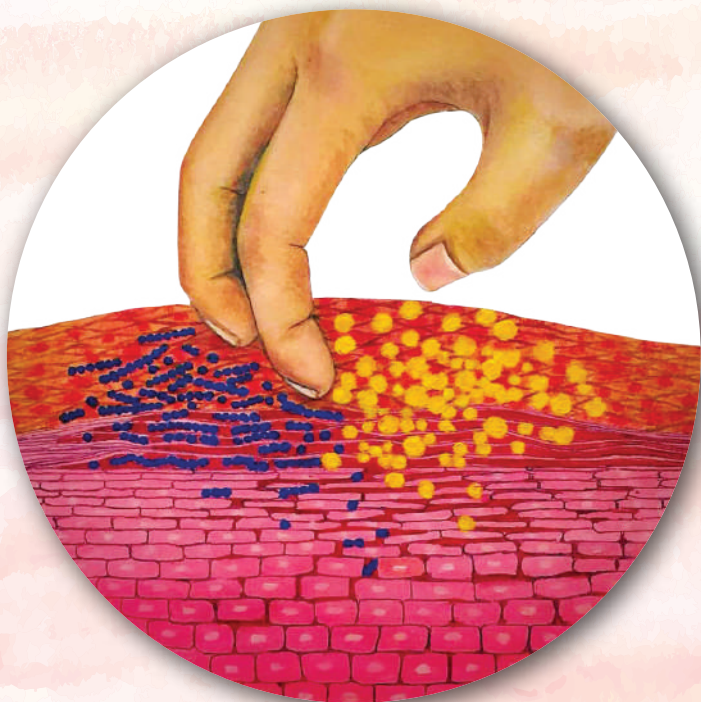


QUICK REFERENCE FOR HEALTHCARE PROVIDERS

MANAGEMENT OF ATOPIC ECZEMA



Ministry of Health
Malaysia



Persatuan Dermatologi
Malaysia



Academy of
Medicine Malaysia

KEY MESSAGES

1. Atopic eczema (AE) is a clinical diagnosis based on the U.K. Working Party's Diagnostic Criteria for Atopic Dermatitis (refer to **Table 1**).
2. Serum immunoglobulin E levels, patch test, skin prick test and skin biopsy should not be used as diagnostic tools for AE.
3. Management of AE depends on the disease severity and quality of life assessment (refer to **Algorithm 1** and **2**).
4. Emollient therapy is the mainstay of treatment at any stage of AE.
5. Topical corticosteroids (TCS) should be used appropriately for treatment of flares in AE (refer to **Table 3**).
6. Topical calcineurin inhibitors may be considered to treat flares in AE for patients aged two years and above.
7. Ultraviolet A1 may be used to control acute flares and narrow-band ultraviolet B in moderate to severe chronic AE.
8. Antihistamines should not be used as monotherapy or to substitute topical therapy in AE.
9. Systemic corticosteroids may be considered for short-term control of severe acute exacerbation of AE.
10. Immunomodulating agents such as azathioprine, cyclosporin A, methotrexate or mycophenolate may be used in the treatment of severe AE after optimisation of topical treatment.
11. Educational interventions should be considered as part of the management of AE (refer to **Table 4**).

This Quick Reference provides key messages and a summary of the main recommendations in the Clinical Practice Guidelines (CPG) Management of Atopic Eczema.

Details of the evidence supporting these recommendations can be found in the above CPG, available on the following websites:

Ministry of Health Malaysia : www.moh.gov.my

Academy of Medicine Malaysia : www.acadmed.org.my

Persatuan Dermatologi Malaysia : www.dermatology.org.my

CLINICAL PRACTICE GUIDELINES SECRETARIAT

Malaysian Health Technology Assessment Section (MaHTAS)

Medical Development Division, Ministry of Health Malaysia

Level 4, Block E1, Precinct 1,

Federal Government Administrative Centre 62590

Putrajaya, Malaysia

Tel: 603-88831229

E-mail: htamalaysia@moh.gov.my

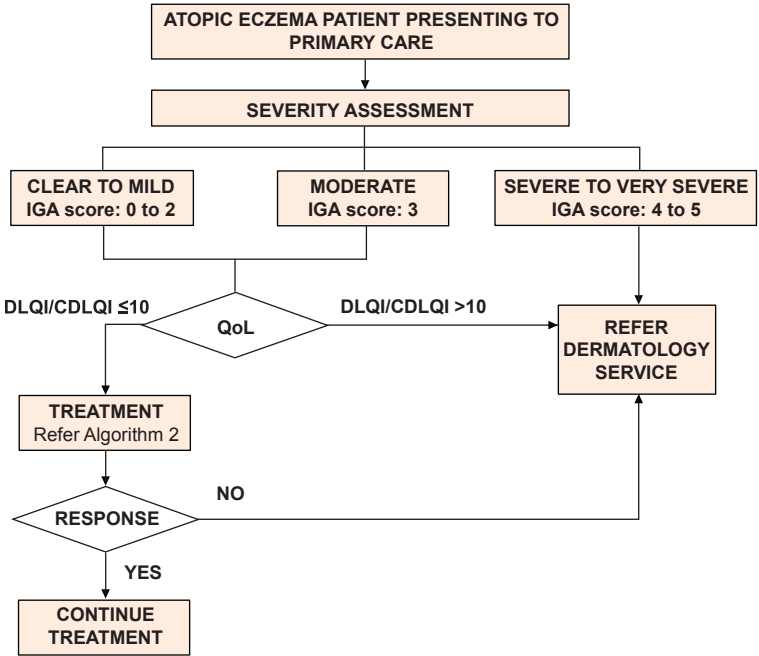
TABLE 1. THE U.K. WORKING PARTY’S DIAGNOSTIC CRITERIA FOR ATOPIC DERMATITIS

THE U.K. WORKING PARTY’S DIAGNOSTIC CRITERIA FOR ATOPIC DERMATITIS
Patient must have an itchy skin condition (or parental report of scratching or rubbing in a child) plus 3 or more of the following: • history of involvement of the skin creases e.g. folds of elbows, behind the knees, fronts of ankles or around the neck (including cheeks in children under 10 years old) • a personal history of asthma or hay fever (or history of atopic disease in a first-degree relative in children under 4 years old) • a history of a general dry skin in the last year • visible flexural eczema (or eczema involving the cheeks/forehead and outer limbs in children under 4 years old) • onset under the age of 2 (not used if child is under 4 years old)

TABLE 2. CRITERIA FOR REFERRAL

CRITERIA FOR REFERRAL
The urgency for referral to a dermatologist is divided into the following categories: 1. Urgent referral (within 24 hours) • AE with clinical suspicion of eczema herpeticum (eczema with widespread herpes simplex infection) • AE with severe skin bacterial infection that requires intravenous antibiotics • AE with acute erythroderma where the eczema is affecting >80% body surface area 2. Non-urgent referral • Diagnostic uncertainty • Severe or uncontrolled eczema: ◦ requirement of potent and very potent TCS ◦ frequent infections ◦ poor sleep or excessive scratching ◦ treatment failure with appropriate topical therapy regimen • Parental concern • Need for treatment demonstration/education • Involvement of sites that are difficult to treat • Psychological disturbance on the patient or family

ALGORITHM 1. MANAGEMENT OF ATOPIC ECZEMA IN PRIMARY CARE



Investigator’s Global Assessment

Score	Description
0 = Clear	No inflammatory signs of atopic eczema
1 = Almost clear	Just perceptible erythema, and just perceptible papulation/infiltration
2 = Mild disease	Mild erythema, and mild papulation/infiltration
3 = Moderate disease	Moderate erythema, and moderate papulation/infiltration
4 = Severe disease	Severe erythema, and severe papulation/infiltration
5 = Very severe disease	Severe erythema, and severe papulation/infiltration with oozing/crusting

IGA : Investigators’ Global Assessment
QoL : Quality of life
DLQI : Dermatology Life Quality Index
CDLQI: Children’s Dermatology Life Quality Index

ALGORITHM 2. TREATMENT OF ATOPIC ECZEMA

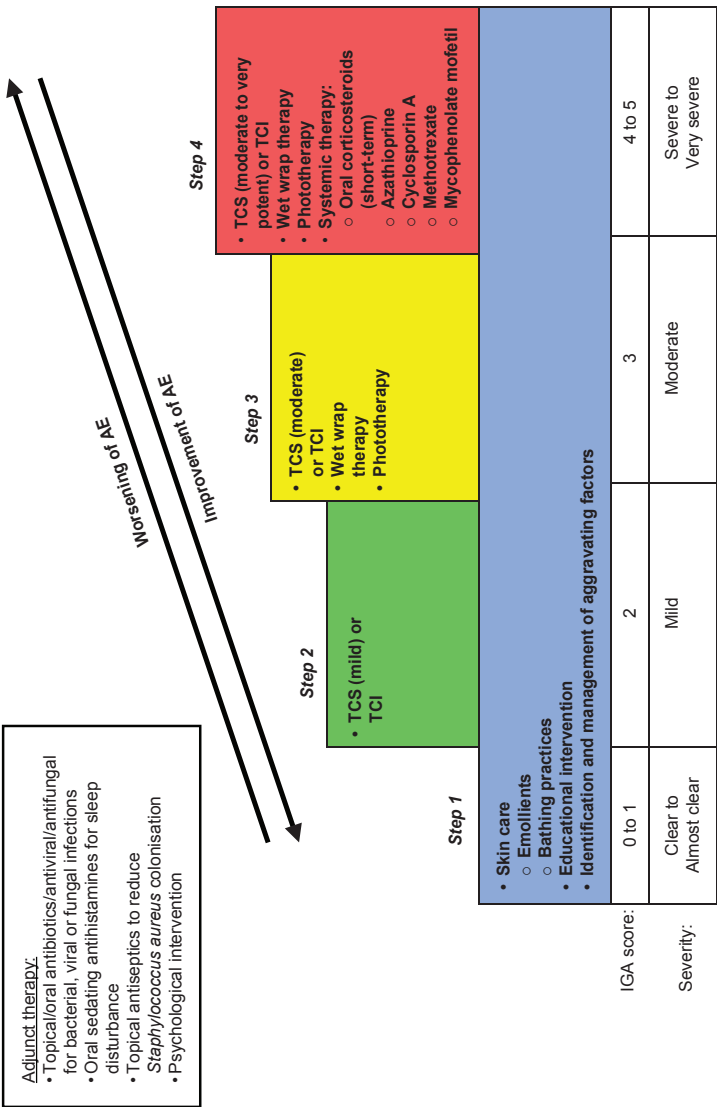


TABLE 3. RECOMMENDED TOPICAL CORTICOSTEROIDS DOSING, SIDE EFFECTS AND CONTRAINDICATIONS

DRUG	RECOMMENDED DOSAGE	POSSIBLE SIDE EFFECTS	CONTRAINDICATION	SPECIAL PRECAUTIONS
TOPICAL CORTICOSTEROIDS				
Mild Betamethasone Valerate 1 in 10 dilution (0.01%) Cream/Ointment	1 - 2 times daily	Worsening of untreated infection, contact dermatitis, perioral dermatitis, acne, depigmentation, dryness, hypertrichosis, secondary infection, skin atrophy, pruritus, tingling/stinging, rosacea, folliculitis, photosensitivity	Untreated bacterial, fungal or viral skin lesions in rosacea and perioral dermatitis	Avoid prolonged use Caution when used on face or intertriginous and flexor areas
Betamethasone Valerate 1 in 8 dilution (0.0125%) Cream/Ointment				
Hydrocortisone Acetate 1% Cream/Ointment				
Moderate Betamethasone Valerate 1 in 2 dilution (0.05%) Cream /Ointment	1 - 2 times daily			
Betamethasone Valerate 1 in 4 dilution (0.025%) Cream/Ointment				
Clobetasone Butyrate 0.05% Cream/Ointment				

TABLE 3. RECOMMENDED TOPICAL CORTICOSTEROIDS DOSING, SIDE EFFECTS AND CONTRAINDICATIONS

DRUG	RECOMMENDED DOSAGE	POSSIBLE SIDE EFFECTS	CONTRAINDICATION	SPECIAL PRECAUTIONS
TOPICAL CORTICOSTEROIDS				
Potent Betamethasone Dipropionate 0.05% Cream /Ointment	1 - 2 times daily	Worsening of untreated infection; contact dermatitis, perioral dermatitis, acne, depigmentation, dryness, hypertrichosis, secondary infection, skin atrophy, pruritus, tingling/stinging, rosacea, folliculitis, photosensitivity	Untreated bacterial, fungal or viral skin lesions in rosacea and perioral dermatitis	Avoid prolonged use Caution when used on face or intertriginous and flexor areas
Betamethasone Valerate 0.1% Cream /Ointment				
Fluocinolone Acetonide 0.025% Cream				
Fluticasone Propionate 0.05% Cream				
Triamcinolone Acetonide 0.1% Cream	Once daily			
Mometasone Furoate 0.1% Cream/Ointment				
Very Potent Clobetasol Propionate 0.05% Cream/Ointment	1 - 2 times daily			

TABLE 4. WRITTEN ECZEMA ACTION PLAN

NAME:	GREEN = GO : Use preventive measures YELLOW = CAUTION: Use lower strength medications RED = FLARE : Use higher strength medications and consult your doctor
GREEN 	ECZEMA UNDER CONTROL REGULAR DAILY SKIN CARE 1. Bath twice a day with gentle cleanser less than 10 minutes. 2. Apply moisturiser to all body parts immediately after bath. 3. Apply moisturiser to all body parts minimum thrice a day. 4. Bath and moisturise your skin before bed. 5. Wear suitable cloth/pyjamas, preferably cotton, to bed.
YELLOW 	ECZEMA WORSENING SKIN CARE DURING WORSENING 1. Continue regular skin care from GREEN phase. 2. Apply anti-inflammatory creams till eczema clears. 2a. Face: Apply hydrocortisone 1% twice a day for 5 - 7 days, then once a day for 5 - 7 days till eczema clears. 2b. Body: Apply betamethasone (1:4) twice a day for 5 - 7 days, then once a day for 5 - 7 days till eczema clears. 3. Take antihistamine (anti-itch), prescribed by doctor, half an hour before bed. 4. If eczema gets better, revert back to GREEN phase. 5. If eczema not responding within 3 days or eczema and itch worsens, move to RED phase.
RED 	UNCONTROLLED ECZEMA SKIN CARE DURING UNCONTROLLED ECZEMA 1. Continue regular skin care form GREEN phase. 2. Bath daily with antiseptic wash for 5 - 7 days. 3. Apply anti-inflammatory creams till eczema clears. 3a. Face: Apply betamethasone (1:8) twice a day for 5 - 7 days, then once a day for 5 - 7 days till eczema clears. 3b. Body: Apply betamethasone (1:2) twice a day for 5 - 7 days, then once a day for 5 - 7 days till eczema clears. 4. Take antihistamine (anti-itch), prescribed by doctor, half an hour before bed. 5. If eczema gets better revert back to YELLOW phase, then subsequently to GREEN phase. 6. If eczema not responding within 3 days or eczema and itch worsens, consult your doctor.